



Alberta Public Health Disease Management Guidelines

Chancroid



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Health and Wellness Promotion Branch

Public Health and Compliance Branch

Alberta Health

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Case Definition

Confirmed Case

Laboratory confirmation of infection with clinical illness.^(A)

- Detection of *Haemophilus ducreyi* nucleic acid (PCR) from an appropriate clinical specimen^(B) (e.g., genital ulcer swab, bubo aspirate).

^(A) Clinical illness is characterized by a single or multiple painful necrotic ulcers that bleed on contact; often accompanied by painful, swollen and suppurating regional lymph nodes.

^(B) Refer to the [National Microbiology Laboratory Guide to Services](#) for current specimen collection and submission information.

Reporting Requirements

Physicians, Health Practitioners and Others

Note: This section includes First Nations and Inuit Health Branch

- Physicians, health practitioners and others shall notify the Sexually Transmitted Infection (STI) Medical Director^(C) via Sexually Transmitted Infection Centralized Services (STICS), of all confirmed cases within 48 hours (two business days) by forwarding a completed [Notification of STI](#) form.

Laboratories

- All laboratories shall report all positive laboratory results by mail, fax or electronic transfer within 48 hours (two business days) to the STI Medical director via STICS and the Chief Medical Officer of Health (CMOH) (or designate).

Alberta Health Services - STICS

Contact Information: Toll free: 1-855-945-6700 option 4
Fax: 780-670-3624

The STI Medical Director/STICS shall forward the initial Notification of STI form of all confirmed cases to the CMOH (or designate) within two weeks of notification and the final Notification of STI form within four weeks. The STI Medical Director/STICS are responsible for ensuring investigation and follow-up of all reported confirmed cases.

- For out-of-province and out-of-country reports, the following information (when available) should be forwarded to the CMOH (or designate) by phone, fax or secure/encrypted electronic transfer as soon as possible:
 - name,
 - date of birth,
 - out-of-province health care number,
 - out-of-province address and phone number,
 - positive laboratory report, and
 - other relevant clinical/epidemiological information.
- For out-of-province and out-of-country contacts the following information (when available) should be forwarded to the CMOH (or designate) as soon as possible:
 - name,
 - date of birth,
 - date of exposure, and
 - out-of-province/country contact information.

^(C) The STI Medical Director is the Provincial Medical Director of Alberta Health Services' Sexually Transmitted Infection Centralized Services (STICS) and is also a Medical Officer of Health.

Additional Reporting Requirements for Physicians, Health Practitioners and Others

In all cases, where a person under 18 is suspected or confirmed to have a STI, an assessment should be carried out by the clinician to determine if additional reporting is required.

To Alberta Child and Family Services

- The clinician should determine whether there are reasonable and probable grounds to believe that they are in contact with “a child in need of intervention” (as per Section 1(2) of the [Child, Youth and Family Enhancement \(CYFE\) Act](#)) and shall report to a director pursuant to Section 4 of the *CYFE Act*.⁽¹⁾
- Reporting is done by contacting the local Child and Family Services office or calling the **CHILD ABUSE HOTLINE: 1-800-387-5437 (KIDS)**. For local office contact information see [the ministry’s website](#).

To Law Enforcement Agency

- Consent is a key factor in determining whether any form of sexual activity is a criminal offence. Children under 12 do not have the legal capacity to consent to any form of sexual activity. The law recognizes that the age of consent for sexual activity is 16. However, the law identifies the exception for minors between 12 and 16 years as having the ability to consent, in “close in age” or “peer group” situations.⁽²⁾
- Reporting is done by contacting your local City Police Detachment or [RCMP Detachment](#).
- For additional information:
 - Alberta [Child, Youth and Family Enhancement Act](#)
 - Age of Consent to Sexual Activity at www.justice.gc.ca/eng/rp-pr/other-autre/clp/faq.html
 - Criminal Code of Canada at www.laws-lois.justice.gc.ca/eng/acts/C-46/⁽³⁾

Epidemiology

Etiology

Chancroid is an acute bacterial infection caused by *Haemophilus ducreyi*, a gram-negative coccobacillus.⁽⁴⁾

Clinical Presentation

This ulcerative disease is generally found in the genital area. The infection begins as a tender erythematous papule four to seven days after infection. The papule becomes pustular and breaks down over several days into a shallow painful ulcer with ragged edges and a red border or halo. One or more of the necrotizing ulcers may join together. The deeper ulcers may lead to marked tissue destruction. In women, lesions may occasionally be present on the vaginal wall or cervix. Lymph nodes in the area (usually unilateral) become swollen, painful, and matted together, forming an abscess or bubo in the groin. The skin may become red and shiny and eventually break down to form a sinus. Auto-inoculation may result in new lesions forming.^(4,5)

Chancroid ulcers increase the risk of human immunodeficiency virus (HIV) infection. Co-infection with syphilis or herpes simplex virus is common.⁽⁶⁾

Diagnosis

Diagnosis is made based on the appearance of the ulcers/lymph nodes and laboratory testing of a sample of discharge or pus. This is generally made by excluding other infections that are associated with genital ulcer disease such as herpes, syphilis or lymphogranuloma venereum. The Public Health Laboratory (ProvLab) should be consulted in advance to optimize specimen preparation and culture results. The prescribed guidelines for the laboratory should be followed.⁽⁷⁾

Treatment

Reference 6 applies to this section.

- Treatment should be initiated after consultation with an STI specialist and may include:
 - ciprofloxacin 500 mg orally x single dose (A-1), OR
 - azithromycin 1 g orally x single dose (A-1), OR
 - ceftriaxone 250 mg intramuscularly x single dose (A-1).
- Treatment failures may occur, especially in HIV co-infected patients. All treatment failures should be carefully evaluated with respect to both the etiology and the possible co-existence of other pathogens. Buboec should be aspirated or incised to relieve pain and prevent spontaneous rupture.

Pediatric Cases

- Neonates born to untreated, infected mothers must be tested and treated.
- When a case is diagnosed in an infant, the mother and her sexual partner(s) should be examined and tested.
- It is recommended that all children < 14 years of age be referred to a pediatrician and, because of the high risk of sexual abuse, be managed in consultation with one of the following referral centres.

Edmonton

Child and Adolescent Protection Centre
Stollery Children's Hospital
1C4.24 Mackenzie Health Sciences Centre
8440-112 Street
Edmonton, Alberta T6G 2B7
Tel: 780-407-1240

Calgary

Child Abuse Service
Child Development Centre
Suite 200, 3820-24 Ave NW
Calgary, Alberta. T2N 1N4
Tel: 403-955-5959

Reservoir

Humans are the only known reservoir.⁽⁸⁾

Transmission

This bacterial infection is transmitted person to person through direct contact with discharges from lesions and pus.⁽⁸⁾ Auto-inoculation to non-genital sites may take place. Asymptomatic carriage can occur.

Incubation Period

The incubation period generally lasts three to five days but may extend to 14 days.⁽⁸⁾

Period of Communicability

The infection may be passed until lesions or buboes are healed. Without treatment, the infectious agent may persist in the lesion or discharging lymph nodes for several weeks or months. Treatment with antibiotics eradicates *H. ducreyi*, and lesions generally heal in one to two weeks.⁽⁸⁾

Host Susceptibility

Susceptibility is general and there is no evidence of natural resistance. Uncircumcised males are at higher risk of infection than circumcised males.⁽⁸⁾ Chancroid ulcers are associated with an increased risk of acquiring HIV.⁽⁶⁾

Incidence

General

Chancroid is most prevalent in tropical and subtropical regions of the world. It is less common in temperate zones but is considered to have worldwide distribution. Infection is generally associated with poverty, poor hygienic conditions, urban prostitution and illicit drug use. It may be endemic in some locations.⁽⁹⁾

Canada

Occurrence in Canada is rare. Outbreaks in the late 1970s and early 1980s occurred but numbers are generally low. Chancroid has not been a nationally reportable infection since 2000.⁽¹⁰⁾

Alberta

No new cases of Chancroid have been reported between 1998 and 2012.⁽¹¹⁾

Refer to the [Interactive Health Data Application](#) for more information.

Public Health Management

Key Investigation

The diagnosis and treatment is performed by community physicians (in the majority of cases) and STI Clinics (Edmonton, Calgary, Fort McMurray). The diagnosis and treatment is performed by community health care providers.

- Determine the presence or absence of symptoms.
- Determine if risk factors for chancroid are present:
 - a history of relevant exposure including immigrating from or having sex with partners from a chancroid endemic country,
 - previously undiagnosed symptoms, and/or
 - similar illness among recent partners.
- Offer testing for HIV and other STI.
- Counsel and identify partners, including locating information.

Management of a Case

- Cases should be interviewed for history of exposure, risk assessment, and sexual partner(s) identification.
- If genital ulcers are present, test for herpes, syphilis, and consider testing for lymphogranuloma venereum and/or granuloma inguinale.
- All cases should be instructed about infection transmission. Patients should be counseled about the importance of abstaining from sex until appropriate diagnosis and treatment is completed.
- All cases should be provided with individualized STI prevention education, targeted at developing knowledge, skills, attitudes and behaviors to reduce the risk and prevent recurrences of STI.
- Immunization against hepatitis A may be recommended. Refer to the [Alberta Immunization Policy](#) for immunization eligibility.⁽¹²⁾
- Immunization against hepatitis B is recommended if not already given. Refer to Alberta Immunization Policy for immunization eligibility.⁽¹²⁾
- All patients with a notifiable STI qualify for provincially funded medications.
 - STI Services will send replacement medication upon receipt of a Notification of STI Form when the physician mailing address is indicated on the form.
 - Physicians and STI clinics may order additional quantities of medication by contacting STI Services.
- For advice on management of your case, call **STICS toll free 1-855-945-6700, option 4.**
- Sexual assault in adults should be managed in conjunction with local Sexual Assault Services and other appropriate community support services.

Recalcitrant Patients

The [Public Health Act](#) (sections 39 through 52) authorizes detention of recalcitrant patients for medical examination, treatment and/or counselling.

- The CMOH (or designate) or MOH (or designate) may issue a certificate to detain an individual who is believed to be infected and refuses or neglects to comply with treatment.
- In order to enact this provision, there must be proof of infection or contact with an infected person and:
 - documentation of failure to comply with prescribed treatment and medical examination, or
 - non-compliance for testing and/or treatment.

Management of Contacts

Partner Notification

- Partner notification will identify those at risk, reduce disease transmission/re-infection and ultimately prevent disease sequelae.
- **It is mandated under the *Communicable Diseases Regulation* that every attempt is made to identify, locate, examine and treat partners/contacts of all cases.**
- All contacts in the 14 days prior to date of initial symptom onset, regardless of signs or symptoms, must be located, examined, tested and treated. Empirical treatment with a quinolone or another antibacterial known to be effective should be provided regardless of clinical findings and without waiting for test results.
- If ulcers are present, investigations similar to the index case should be undertaken.
- Contacts should abstain from unprotected intercourse until the treatment of the case is complete.
- All contacts should be:
 - instructed about infection transmission, and
 - provided with individualized STI prevention education, targeted at developing knowledge, skills, attitudes and behaviors to reduce the risk and prevent recurrences of STI.
- The physician/case manager are required to provide partner names and locating information on the STI Notification Form and forward to STICS.
- If testing and/or treatment of partners are not confirmed on the STI Notification Form, STICS will initiate follow up by a Partner Notification Nurse (PNN).
 - PNNs are specially trained to conduct notification of partners and contacts in a confidential manner that protects the identity of the index case.
 - The phone number for your designated PNN is available by calling **STICS toll free 1-855-945-6700, option 4.**
- STICS will follow-up on all out of province/country referrals of cases and partner(s).

Preventive Measures

- Ensure appropriate treatment of *H. ducreyi* for cases.
- Interview the case; identify and ensure appropriate treatment and follow-up of *H. ducreyi* for sexual partner(s).
- Ensure STI care is culturally appropriate, inclusive, readily accessible, and acceptable.
- Include information about risk for STI during pre-travel health counseling.

- Educate the case, sexual partner(s), and the public about symptoms, transmission and prevention of infection including:⁽⁶⁾
 - personal protective measures including the correct and consistent use of condoms,
 - abstinence,
 - delaying onset of sexual activity,
 - reducing the numbers of sexual partners,
 - developing mutually monogamous relationships,
 - discouraging anonymous or casual sexual activity, and
 - sound decision making.

Appendix 1: Revision History

Revision Date	Document Section	Description of Revision
October 2021	General	• Updated Template • Etiology, Clinical Presentation, Diagnosis and Treatment sections moved to Epidemiology • Key Investigation section moved to Public Health Management (formerly called Control) • Updated/inserted web links
	Reporting Requirements	• Revised to reflect current practice
	Partner Notification	• Revised to reflect current practice

References

- (1) Government of Alberta. Child Youth and Family Enhancement Act Policy Manual. Edmonton: Government of Alberta; 2010 Nov 1.
- (2) Government of Canada. Age of Consent to Sexual Activity. 2011 May 12 Available from: URL: <http://www.justice.gc.ca/eng/dept-min/clp/faq.html>
- (3) Government of Canada. Criminal Code of Canada. 2011
- (4) Spinola SM. Chancroid and Haemophilus ducreyi. In: Holmes KK, Sparling PF, Stamm WE, Poit P, Wasserheit JN, Corey L, et al., editors. Sexually Transmitted Diseases. 4 ed. New York: McGraw Hill Medical; 2008. p. 689-99.
- (5) American Academy of Pediatrics. Chancroid. In: Pickering LK, Baker CJ, Kimberlin DW, Long SS, editors. Red Book: 2009 Report of the Committee on Infectious Diseases. 28 ed. Grove Village, IL.: American Academy of Pediatrics; 2009. p. 250-2.
- (6) Public Health Agency of Canada. Chancroid. 2010
- (7) Centers for Disease Control and Prevention. Sexually Transmitted Diseases Treatment Guidelines, 2010. Morbidity and Mortality Weekly Report 2010 December 17; 59(No. RR-12):1-110.
- (8) Ballard R, Ndowa F, Tun Y. Chancroid. In: Heymann DL, editor. Control of Communicable Diseases Manual. 19 ed. Washington, DC: American Public Health Association; 2008. p. 107-9.
- (9) American Academy of Pediatrics. Chancroid. In: Pickering LK, Baker CJ, Kimberlin DW, Long SS, editors. Red Book: 2009 Report of the Committee on Infectious Diseases. 28 ed. Grove Village, IL.: American Academy of Pediatrics; 2009. p. 250-2.
- (10) Public Health Agency of Canada. Report on Sexually Transmitted Infections in Canada: 2008. Public Health Agency of Canada 2009 March 1 Available from: URL: <http://www.phac-aspc.gc.ca/std-mts/report/sti-its2008/05-eng.php>
- (11) Communicable Disease Reporting System (CDRS) [computer program]. 2012.
- (12) Government of Alberta. Alberta Immunization Policy. Edmonton; 2012.